



Medication Guideline

Pantoprazole Intravenous

Reference #: MMG046

Scope

Site	Service/Department/Unit	Disciplines
SCGOPHCG	Organisation-wide	Medical, Nursing, Pharmacy

Introduction¹⁻²

Pantoprazole is a proton pump inhibitor (PPI) which binds to the hydrogen/potassium ATPase enzyme system, inhibiting both stimulated and basal acid secretion.

Treatment with pantoprazole causes reduced acidity in the stomach and thereby an increase in gastrin in proportion to the reduction in acidity. The increase in gastrin is reversible.

Availability¹⁻²

1. Pantoprazole sodium 40mg Vial

Contraindications²

2. Known hypersensitivity to any components of the formulation. Esomeprazole IV is available for patients with hypersensitivity to pantoprazole.

Precautions¹⁻²

3. Cirrhosis or severe liver disease – risk of accumulation when higher doses are used. Consider risk versus benefit approach in patients with hepatic impairment.

Indications¹⁻³

- Upper gastrointestinal bleed
- Stress ulcer prophylaxis in patients who are nil by mouth
- Zollinger-Ellison syndrome
- Duodenal ulcer
- Erosive esophagitis
- Gastric hypersecretion
- General Surgery (Ileostomies / Ivor Lewis's / Whipples)
- Peptic ulcer disease

Dosage¹⁻⁵

- **Upper Gastrointestinal Bleed**
80mg as an infusion over 15-30 minutes, then either a continuous infusion at 8mg/hour for up to 72 hours or 40mg TWICE daily for up to 72 hours.
Change to an oral twice daily PPI as soon as clinically appropriate.
Initial oral therapy may be appropriate for patients at low risk of re-bleeding.
- **Stress ulcer prophylaxis in patients with nil by mouth**
40mg IV once daily
- **Zollinger-Ellison syndrome**
80mg IV 2 or 3 times daily (change to oral treatment as soon as possible – 80mg – 120mg twice daily or 80mg three times daily).
- **Duodenal ulcer**
40mg IV once daily
- **Erosive esophagitis**
40mg IV once daily for 7 to 10 days
- **Gastric hypersecretion**
80mg IV twice daily
- **General surgery**
40mg IV once daily
- **Peptic Ulcer Disease (PUD)**
40 mg IV once daily for 4–8 weeks; change from IV to oral treatment as soon as possible.

Preparation & Administration⁶

IV Bolus

Dissolve the contents of each 40mg vial with 10mL of Sodium Chloride 0.9%. Administer the contents of each vial over 2 minutes (if multiple vials are required for the dose, it may be easier to administer as an infusion).

IV Infusion

Intermittent Infusion:

Dissolve the contents of each 40mg vial with 10mL of Sodium Chloride 0.9%, then dilute the prescribed dose to 100mL with compatible fluid and infuse over 15 minutes.

Continuous Infusion:

Dilute 80mg (2 vials) to 100mL of compatible fluid and infuse over 10 hours (i.e. 8mg/hr).

Compatible Fluids⁶

Fluids: Sodium Chloride 0.9%, Glucose 5%, Glucose 10% (Somac only), Hartmann's (Somac only).

Y-site: Pantoprazole sodium compatibility is concentration dependent. Do not mix undiluted pantoprazole with any other drugs.

Incompatible Fluids⁶

Fluids: No information.

Drugs: Do not mix undiluted pantoprazole with any other drugs.

Pantoprazole sodium compatibility is concentration dependent.

Storage⁶

- Store vials below 25 °C. Protect from light (diluted solutions do not need to be protected from light).
- The reconstituted solution is stable for 12 hours at 2 to 8 °C.

Practical Points^{1-2,7}

Pregnancy and Breastfeeding

- Consider alternative medication such as omeprazole. (see [King Edward Memorial Hospital Obstetrics & Gynaecology – Pantoprazole](#)).
- Breastfeeding – considered safe to use.

Adverse effects

- Injection site reactions
- Headache
- Dizziness
- Nausea/vomiting

Other

- If patient is already taking an oral PPI prior to admission, it is suitable to continue the same PPI at an appropriate dose on discharge.

Related Documents

Legislation:

- [Medicines and Poisons Act 2014](#)
- [Medicines and Poisons Regulations 2016](#)
- [Health Services Act 2016](#)

Policy:

- [Medicines Management Policy and Procedure SCGOPHCG](#)
- [Mandatory Policy MP0077/18 Statewide Medicines Formulary Policy](#)

Guidelines:

- [Nursing Practice Guidelines No 4: Intravenous Therapy](#)
- [King Edward Memorial Hospital Obstetrics & Gynaecology – Pantoprazole](#)

References

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